Thyroid Examination — OSCE Checklist

Deakin MD2 · ELC5 · Time: 8 min (+2 min reading)

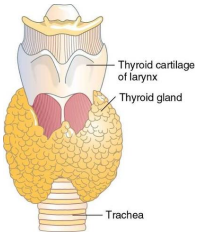
Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **General → Hands → Vitals/Arms → Eyes/Face → Neck → Chest → Legs**. *Look for* **hyper- AND hypothyroid** *signs at every step and say what you do / don't see.*

1. Introduction

☐	**W**ash hands
☐	**I**ntroduce self (name, role)
☐	**P**atient ID: name and DOB
☐	**E**xplain the exam and gain consent
☐	**E**xpose neck to clavicles; **P**osition seated on a chair (room to stand behind)
☐	Ask about pain / neck tenderness; have a glass of water ready

2. Examination

	Step	What to do / say	Abnormal
General inspection			
☐	Body habitus	Signs of weight change	Loss (hyper); gain not necessarily more common (hypo)
☐	Clothing	Dressed for the weather? Heat / cold intolerance	Layers = hypo; sweaty, few clothes = hyper
☐	Well / unwell	Anxiety, agitation, restlessness	Hyper
☐	Obvious neck mass	Goitre or thyroglossal cyst	May be seen before regional exam
Hands			
☐	Nails	Clubbing (thyroid acropachy), onycholysis	Hyper (Graves)
☐	Skin	Palmar erythema, warm, sweaty	Hyper
☐	Skin	Peripheral cyanosis, palmar pallor, yellowing (hypercarotenaemia)	Hypo
☐	Fine tremor	Arms outstretched, sheet of paper on the backs of hands	Hyper (sympathetic overactivity)
Vital signs and arms			
☐	Radial pulse (manual)	Rate, rhythm, volume	Tachycardia / AF (hyper — confirm on ECG); slow, small volume (hypo)
☐	Vital signs	BP, RR, SpO_2, temp	Wide pulse pressure, fever (hyper) †
☐	Arm reflexes	Biceps, brachioradialis	Brisk (hyper); slow relaxation (hypo) †
☐	Proximal myopathy	Arms overhead / resisted "chicken wings" (can do with legs)	Hypo or hyper
Eyes and face			
☐	Face / skin	Pallor, yellowing, thickened skin, alopecia, vitiligo, xanthelasma	Hypo; vitiligo = autoimmune
☐	Periorbital / eyebrows	Periorbital oedema; loss of outer third of eyebrows	Hypo
☐	Tongue / speech	Macroglossia, hoarse or slow speech	Hypo
☐	Exophthalmos	Look from front, side and above/behind	Graves' disease only
☐	Lid retraction / stare	Sclera visible above iris	Hyper
☐	Lid lag	Follow finger from high to low	Hyper (sympathetic, Müller's muscle)
☐	Conjunctiva / cornea	Chemosis, conjunctivitis, corneal ulceration	Graves (can't close eyes)
☐	Eye movements + ptosis	H test, ask about diplopia	Ophthalmoplegia (Graves); ptosis — consider myasthenia
Neck — inspect (front and sides)			
☐	Scars, redness, veins	Thyroidectomy scar; redness (thyroiditis); prominent veins	Retrosternal goitre
☐	Swelling	Note any goitre or mass	Diffuse vs nodular
☐	Swallow	"Take a sip of water and hold it… now swallow"	Goitre and thyroglossal cyst move up
☐	Tongue protrusion	"Stick out your tongue"	Thyroglossal cyst moves up; goitre does not
Neck — palpate (from behind)			
☐	Thyroid	Warn patient; both lobes + isthmus: size, shape, consistency, symmetry, tenderness, nodules	Diffuse (Graves), multinodular, solitary nodule
☐	Swallow	Palpate while patient swallows water	Tethering = possible malignancy
☐	Lymph nodes	Cervical and supraclavicular chains	Malignancy
☐	Trachea*	Position †	Deviation from large goitre



Thyroid gland anatomy

Neck — percuss and auscultate			
☐	Percuss	Over upper sternum	Dullness = retrosternal goitre
☐	Pemberton's sign	Arms up beside ears for 1 min: facial plethora, cyanosis, neck vein distension, stridor	Thoracic inlet obstruction
☐	Auscultate	Over each lobe with diaphragm for a bruit	Bruit = hyper (Graves)
Chest			
☐	Gynaecomastia	If not already seen	Hyperthyroidism, hypogonadism
☐	Heart / lungs	Auscultate for pericardial and pleural effusions	Hypo
Legs			
☐	Pretibial myxoedema	Shins: raised, waxy, orange-peel plaques	Graves
☐	Oedema	Press over shin	Non-pitting = hypo
☐	Proximal myopathy	Stand from chair, arms crossed	Hypo or hyper
☐	Reflexes	Knee, ankle	Brisk (hyper); slow relaxing (hypo)

3. Closing and presenting

☐	Thank patient, help them dress, wash hands
☐	Completion: "I would check TFTs (TSH, free T4 ± T3), thyroid antibodies, and arrange a thyroid ultrasound if a goitre or nodule is found." †
☐	Present: state thyroid status first, then the neck: "Mrs X appears clinically euthyroid. There is no goitre, the neck is non-tender with no lymphadenopathy, and no eye signs of Graves' disease."

4. Quick reference

Region	Hyperthyroid	Hypothyroid
General	Weight loss, anxious, agitated, heat intolerance	Weight gain, slow, cold intolerance, dressed warmly
Hands	Sweaty, warm, palmar erythema, onycholysis, acropachy, fine tremor	Cool, dry, pale, yellow (carotenaemia), peripheral cyanosis
Pulse / reflexes	Tachycardia, AF; brisk reflexes	Bradycardia, small volume; slow-relaxing reflexes
Eyes / face	Lid retraction, lid lag; Graves: exophthalmos, chemosis, ophthalmoplegia	Periorbital oedema, loss of outer ⅓ eyebrows, coarse thick skin, alopecia
Neck	Goitre ± bruit (Graves)	Goitre (Hashimoto's) or none
Legs	Pretibial myxoedema (Graves), proximal myopathy	Non-pitting oedema, proximal myopathy

- Goitre: 80% euthyroid, 10% hypo, 10% hyper. Isthmus lies over tracheal rings 2–4.
- Only **Graves' disease** causes exophthalmos and pretibial myxoedema; lid lag and stare occur in any hyperthyroidism.
- Pitfalls: palpating from the front only, forgetting the swallow and tongue tests, not commenting on thyroid status, skipping the legs.